Nursing Essentials

Priority: Assess the tracing together with contractions and maternal status. A sudden deterioration needs a prompt response.

FOCUS 1

Read

- **Usual baseline:** 110–160 beats/min. Moderate variability is 6–25 beats/min in amplitude.
- **Category I:** normal baseline and moderate variability, without late or variable decelerations; accelerations may be absent.
- **Trend:** baseline, variability, accelerations, decelerations, and uterine activity—not one isolated number.

FOCUS 2

Connect

- **Early:** gradual, usually mirrors the contraction; commonly related to head compression.
- **Variable:** abrupt decrease; commonly related to cord compression. Evaluate recurrence, depth, duration, and recovery.
- **Late:** gradual decrease with the nadir after the contraction peak; consider impaired uteroplacental oxygen transfer.

FOCUS 3

Respond

- **Assess immediately:** maternal vital signs, position, contractions, medications, and the fetal pattern; notify the appropriate team.
- **Improve reversible factors:** reposition, address hypotension, and reduce / stop oxytocin according to protocol.
- **Escalate persistent abnormalities:** prepare for additional interventions or expedited birth as directed.

Avoid the common traps

Make it stick: A tracing describes fetal response. Nursing care targets the cause: maternal perfusion, uterine activity, cord concerns, or another emerging problem.

Focus	What to know	Nursing application
Tachysystole	more than five contractions in 10 minutes, averaged over 30 minutes; assess the fetal response too.	Assess contraction frequency
Oxygen	do not routinely apply it for an abnormal tracing when maternal oxygenation is normal; treat maternal hypoxemia.	Treat maternal hypoxemia
Acute bradycardia	respond now. Do not wait for a baseline-definition time interval before evaluating a sudden prolonged drop.	Evaluate immediately

RED FLAGS · ESCALATE PROMPTLY

Promptly escalate persistent bradycardia, recurrent concerning decelerations, worsening variability, or maternal instability; address reversible causes while help arrives.